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Name: سارة كمال محمد احمد National ID: 28606102103301	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
 بطاقة تحقيق الشخصية

سارة
 كمال محمد احمد الشريف
 ٢٤ سن الكوثر - طن الاطباء المهنيين
 الدقي - الجيزة

٢٨٦.٦١.٢١.٣٣.١
 ١٩٨٦/١٠/٢٨
 JC4025774


 ٢٠٢١/١٠
 ٢٨٦.٦١.٢١.٣٣.١
 حاصلة على دبلوم تجارة
 أنثى مسلمة متزوجة
 عيد رمضان شعبان على
 البطاقة سارية حتى ٢٧/١٠/٢٨







A-Patient ID:

1- Name:

سارة كمال محمد احمد

Sex:

3- Medical number:

Female

83105229

Old Med:

4309/2022

Patient Age: 37 Y

MDT Form

Family history

Marital status

2a morbidity

Phone

01201041019

DMT

NRS Score

GROUP

Menopausal status

PREMENOPAUSAL

RHEUMATOID
ARTHRITIS
ON MTX
PLACONIL
STEROIDS
STOPPED
BEFORE
STARTING CH

REASON OF MDT

POST OPERATIVE

Date of MDT

2023-07-27

reast Imaging :

Clinical data

Mets Workup :

CTCAP11/2022-->FREE

IX:

OC

GI

ER:

S/S

PR:

7/8

Her 2 Neu :

0

SISH :

Ki67 :

2

N

1

M

0

sonomammography

RT BREAST SPICULATED HYPOECHOIC MASS LESION 23X22 MM
RT AXLN

PATIENT HISTORY

INITIALLY RT CT2N2 MO RECEIVED NACT AC TAXOL
25/6/2023-->RT BC WLE AND SLNB POST CHEMO
PREVIOUSLY DIAGNOSED INVASIVE CARCINOMA
LOCAL DUCTAL CARCINOMA IN SITU
FREE RESECTED MARGIN,,LN 1/8 MICRO METS
AGE YPTIS YPN1MI

PORT

DJUVANT ZOLADEX FEMARA

MDT OUTCOME & PLAN

ICAL ONCOLOGY

ERY

iatric

2023/7/27

SIGNATURE

[Signature]

RADIOLOGY

PATHOLOGY

Nutrition

[Signature]

Patient Name : (الاسم) : ساره محمد احمد الشريفه	Request No. : 13003291
Patient No. : 28606102103301	Request Date : 14/06/2023
Path No. : 1931/23	Verification Date : 25/06/2023
Sex : Female	Financial Category : نقطة الدولة - لمرأة دار السلام
Age : 37 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data :

Right breast cancer post NACT.

Nature of the specimen :

Right wide local excision and sentinel lymph node biopsy specimens collected at 14/6/23.

Gross Examination:

- Right wide local excision specimen measured 5x4.5x1.5 cm , with covering skin ellipse measured 4x1.
- Tumor extent : An ill defined rubbery greyish white tumor bed site (wired and clipped)
- Size: 1x0.5- cm . (totally submitted)
- Distance from margins: 0.5 cm away from deep margin (least margin)
- Sentinel lymph node biopsy specimen: Fibrofatty tissue piece measured 4x3.5 cm entangling 8 LNs up to 1.5x1cm .
- Frozen section:
- Inferior and deep margins are suspicious for lobular neoplasia.
- Free other margins
- SLNs:(1/6) micrometastasis less than 2 mm in largest dimension

Microscopic examination:

- Total tumor regression in a previously diagnosed carcinoma.
- Focal ductal carcinoma insitu.
- Lymph node involvement (1/8) micrometastasis that was seen in frozen section but not in paraffin sections
- *(2 more nodes were found in paraffin sections)
- Resection Margins: Free resected margins
- Histologic response to therapy: Pathologic partial response

Verified by :

dr.esraa magdy

Patient Name : الاسماء / سارة جمال محمد احمد الشريف	Request No. : 13001152
Patient No. : 28606102103301	Request Date : 17/10/2022
Path No. : 3070/22	Verification Date : 25/10/2022
Sex : Female	Financial Category : أوراق ذات السائق - فئة الدولة
Age : 36 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youse

Pathology Report

CLINICAL DATA

Right breast mass

NATURE OF SPECIMEN

Tru cut biopsy, collected at 16/10/2022 , 10:30 am

GROSS EXAMINATION

Multiple grayish white tissue cores up to 2 cm , totally submitted.

MICROSCOPIC EXAMINATION

- * Histological type: Invasive duct carcinoma .
- * Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II
- * DCIS: No
- * Angiolymphatic: Couldn't be detected in the received biopsy

DIAGNOSIS

Right breast mass , Tru cut biopsy:

Invasive duct carcinoma, Grade II .

For further IHC (ER,PR,HER2,Ki67)

Pending next Sunday

Signature

Prof. Dr. Lubna Omar

Dr. Samah Sayed

Verified by :

saamah sayed

Patient Name	الاسم/ سارة فضل محمد احمد الشريف	Request No.	13001152
Patient No.	28606102103301	Request Date	17/10/2022
Path No.	3070/22	Verification Date	13/11/2022
Sex	Female	Financial Category	المرأة - فئة الدولة
Age	36 Y	Department	Out Patient
Referring Doc.		Registered by	marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Prof. Dr. Gamal Abdelaty
Dr. Marwa Mahmoud

Prof. Dr. Lubna Omar
Dr. Samah Sayed

Verified by :
saamah sayed

Printing Date : 13/11/2022 12:55:04

Page 2 of 2

Printed By : saamah sayed

Patient Name	: الأستاذة / سارة كمال محمد أحمد الشريف	Request No.	: 13001152
Patient No.	: 28606102103301	Request Date	: 17/10/2022
Ref No.	: 3070/22	Verification Date	: 13/11/2022
Sex	: Female	Financial Category	: أورام غير المصنوع - فئة الدولة
Age	: 36 Y	Department	: Out Patient
Referring Doc	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

CLINICAL DATA: Right breast mass.
NATURE OF SPECIMEN: Tru cut biopsy, collected at 16/10/2022, 10:30 am.

ROSS EXAMINATION:

Multiple grayish white tissue cores up to 2 cm, totally submitted.
Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2. ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

MICROSCOPIC EXAMINATION

Histological type: Invasive duct carcinoma.
Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade II
DCIS: No
Angioinvasion: Couldn't be detected in the received biopsy
IPT (Positive control showed appropriate reactivity)
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)
- PR: Moderate nuclear reaction in 60% of tumor cells (Allred score 7/8)
- HER2: No membranous reaction in tumor cells (score 0)
- Ki67: 50% nuclear reaction of tumor cells

DIAGNOSIS

Right breast mass. Tru cut biopsy.
Invasive duct carcinoma, Grade II.
IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- HER2: Negative (score 0).
- Ki67: Positive in 50% of tumor cells.

Verified by:
saamah sayed

Printed By: saamah sayed

Printing Date: 13/11/2022 12:55:04

Page 1 of 2



Patient Name: Sara Kamal Mohamed Ahmed Al Sherif	Date : 14/11/2022
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 36 Year(s)
Medical ID:	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Right breast LIQ irregular speculated mass lesion measuring 2.4 X 1.4 cm.
- Right axillary level I and II pathologically enlarged lymph nodes the largest measures 2.3 X 1.6 cm.
- Few bilateral pleural based nodules , the largest is noted at medial segment of middle lung lobe measuring 5 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- Few right iliac wing sclerotic foci mostly bone islands.



Patient Name: Sara Kamal Mohamed Ahmed Al Sherif	Date : 14/11/2022
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 36 Year(s)
Medical ID:	

- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- *Right breast malignant looking mass lesion with pathological ipsilateral axillary lymph nodes as described.*
- *Bilateral few pleural based nodules likely benign in nature for follow up.*
- *No definite hepatic metastatic deposits.*

Dr. Mohammed Mohsen, M.Sc.

Dr. Amr Farouk, M.D.

Patient's Name:

SARAH KAMAL MOHAMED AHMED

Date:

2026-09-02

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Rousey International Egypt

MULTISLICE CT EXAMINATION OF THE CHEST WITHOUT

CONTRAST ADMINISTRATION REVEALED:-

- Right upper and middle lobe subpleural reticulations and ground glass density likely post radiation sequelae.
- Few tiny bilateral scattered pulmonary nodules (2-3 mm in diameter).
- No pulmonary masses, consolidation, collapse or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial effusion.
- Normal plain CT appearance of the heart and great vessels.
- Upper abdominal cuts showed diffuse low parenchymal attenuation.

Opinion:-

❖ Few tiny bilateral scattered pulmonary nodules likely inflammatory for follow up.

Dr.sherif Wagdy

Dr.sherif Wagdy